PARKINOVA

Comprehensive Parkinson's Research Assessment

A. Patient & Assessment Information

Patient Reference	API-TEST-PT
Age	Not provided
Gender	Not provided
Assessment Date	2026-10-27 12:00:00
Selected Modules	Brain MRI Scans, Archimedean Spiral Analysis
Report Reference ID	API-TEST-001

B. Brain MRI Neuroimaging Analysis

Analysis Status	Completed
Model Prediction	No Parkinson's-related pattern detected
Model Architecture	Parkinson CNN (244×244×3)
Input Preprocessing	244×244 Lanczos Resizing, Channel Standardization
Analysis Timestamp	2026-10-27T12:00:00

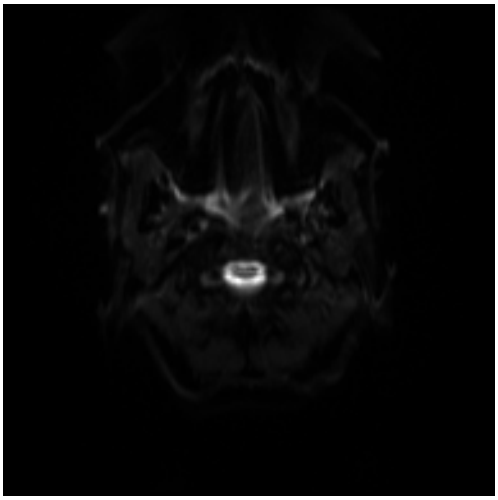
Model Findings & Interpretation:

Test explanation

Model Limitations:

The MRI classifier identifies spatial and intensity patterns correlated with the training cohort. An arbitrary MRI scan alone cannot independently confirm or rule out Parkinson's disease. Model confidence is not clinical probability of disease.

Evaluated Brain MRI Scan:



C. Spiral Drawing Analysis

Spiral Analysis Status	Model Standby (Awaiting Trained Spiral Model File)
Model Prediction	Standby — Integration Interface Ready
Model Name & Version	Spiral Integration v1.0
Image Resolution	400 × 400 pixels
Canvas Surface	Standard
Stroke Density	Analyzed

Visual Observations:

• **Image Geometry:** 400 × 400 pixels

Analysis Limitations & Recommended Next Steps:

Limitations: Static 2D sketch does not replace computerized kinematics.
Next Steps: Consult a neurologist for motor UPDRS evaluation.

D. Combined Multi-Modal Assessment Summary

Single-Active Model Synthesis: MRI deep learning classification was fully completed. The spiral drawing was successfully ingested, validated for geometry and contrast, and placed on standby awaiting trained classification model integration. No synthetic numerical combined risk score is generated to avoid unvalidated risk calculation.

E. Parkinson's Health, Nutrition & Movement Education

Evidence-based supportive lifestyle strategies tailored for individuals with Parkinson's and clinical cohorts:

1. Indian Nutrition & Antioxidants	Focus on neuroprotective, antioxidant-rich foods: cooked spinach (palak), methi, drumstick sambar, ripe papaya, soft ragi kanji/dosas, yellow moong dal khichdi, and soaked almonds. Avoid hard, dry, or splintery textures.
2. Levodopa & Protein Coordination	Dietary large neutral amino acids (LNAAs) compete with levodopa for transport across gut and blood-brain barrier. Take Levodopa 30-60 minutes before or 1-2 hours after protein meals. Do NOT eliminate essential dietary protein.
3. Hydration & Gut Motility	Autonomic constipation affects up to 80% of individuals with Parkinson's. Maintain 6-8 glasses of fluid daily (lukewarm jeera/cumin water, tender coconut water) and adequate soluble fiber (oats, stewed prunes).
4. Safe Physical Activity & Fall Prevention	Daily walking with rhythmic auditory/counting cues (1-2-1-2 cadence), daily calf/chest stretching, supervised balance exercises, and proactive home fall-proofing (removing loose throw rugs, installing night lights).

Regulatory Notice & Academic Disclaimer

This application is an academic AI research prototype. Its output is generated by a machine-learning model and is not a medical diagnosis. The model's performance and reliability may vary depending on image quality, dataset characteristics, and the images used during training. Consult a qualified neurologist for medical evaluation.